

Enhancing Your EHR with PRACX: A Seamless Integration Guide

Connecting the electronic health record to medical billing is essential to an efficient practice. When clinical and financial systems operate independently, staff re-enter the same data, claims are delayed, and reimbursement suffers. PRACX is the billing and integrations layer that sits between the practice EHR and payers.

Audience: PRACX team

Confidential — internal use

September 2026

Key takeaways

- Integrating the EHR with PRACX eliminates duplicate entry of demographics, coverage, and charges, and reduces the transcription errors that drive claim rejections.
- A unified workflow shortens the claim lifecycle: encounter data is validated on intake, scrubbed, submitted electronically or on CMS-1500, acknowledged, posted from ERA, and written back to the EHR as status.
- PRACX is designed to work with the EHR the practice already uses. Current intake supports Elation-oriented PRACX inbound bundle v1 and FHIR R4. Stedi handles 837, acknowledgements, and 835 ERA. Eligibility uses 270/271 with an audited review before coverage or address updates are applied.

Why EHR integration matters in medical billing

When the EHR and billing system do not exchange data, the practice absorbs the inefficiency. Staff manually transfer patient demographics, insurance, encounter details, and codes. The result is wasted time, avoidable denials, and slower cash flow.

Typical consequences of disconnected systems include:

- **Duplicate data entry.** Re-keying names, member identifiers, and diagnoses is a primary source of typos and rejections.
- **Coding discrepancies.** Clinical documentation in the EHR and codes on the claim drift apart, creating compliance risk and denials.

- **Delayed submission.** Manual charge transfer lengthens the interval between date of service and claim filing.
- **Incomplete visibility.** Front desk, clinicians, and billing staff see different versions of coverage and claim status.

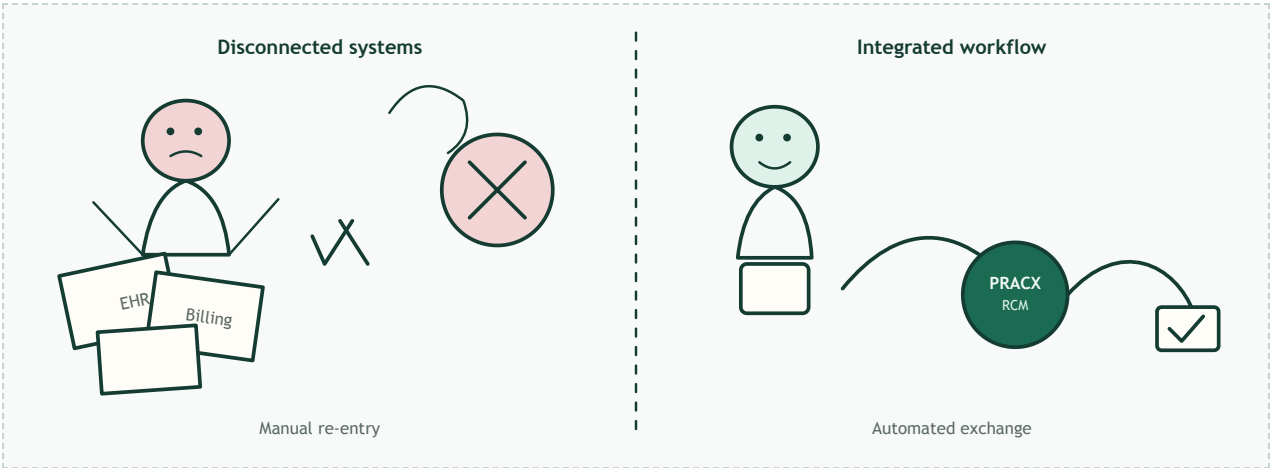
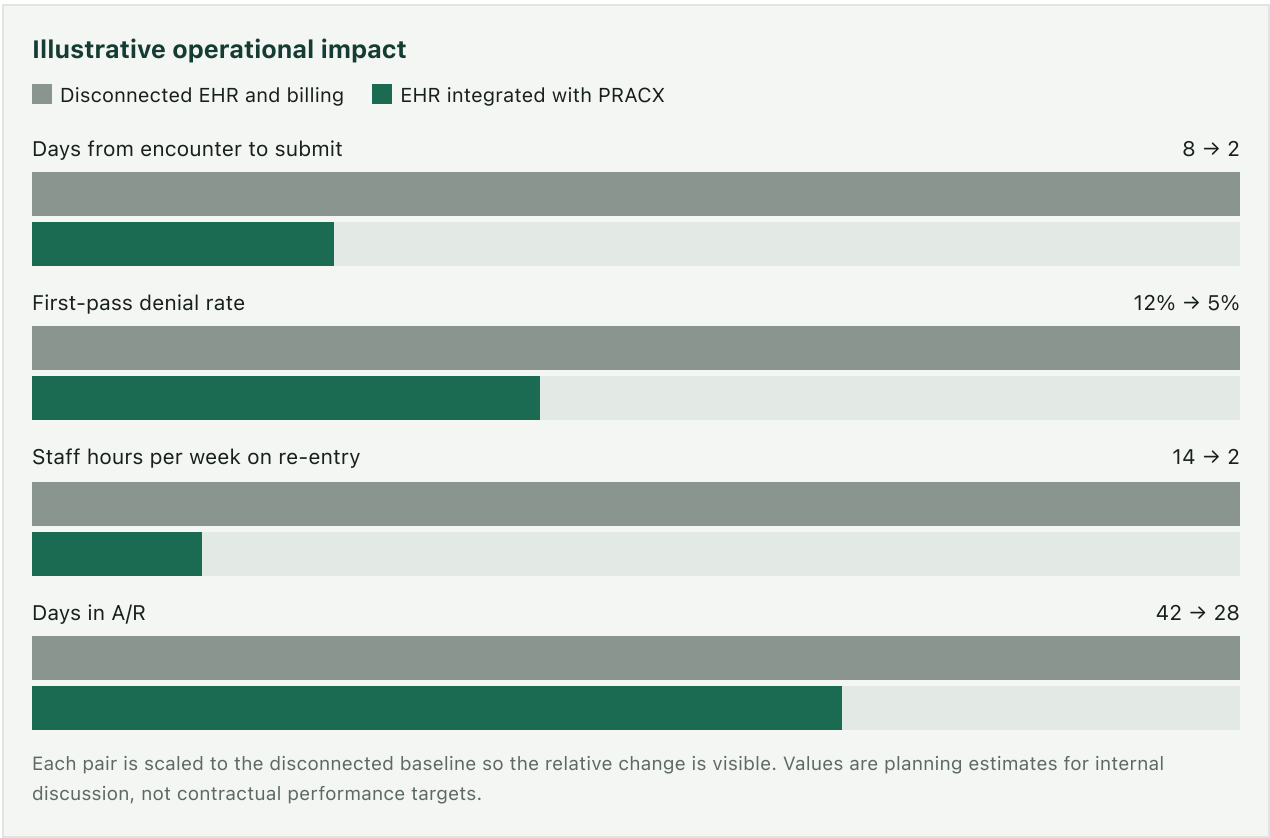
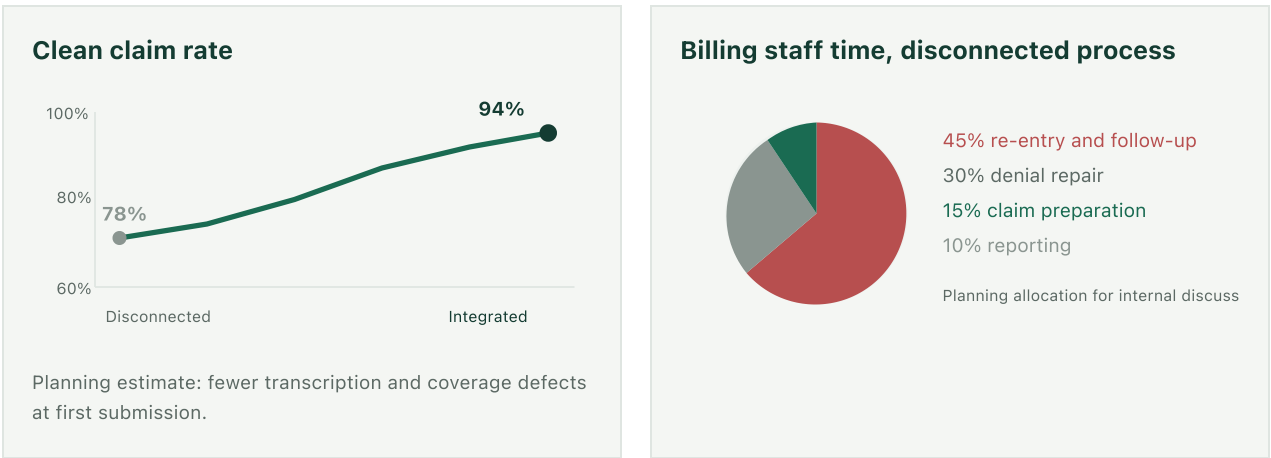


Figure 1. Manual transfer between EHR and billing compared with automated exchange through PRACX.





PRACX architecture

PRACX does not replace the EHR. Clinical documentation remains in the system of care. PRACX receives claim inputs, validates them, prepares and submits claims, posts payments, and returns lifecycle status to the EHR.

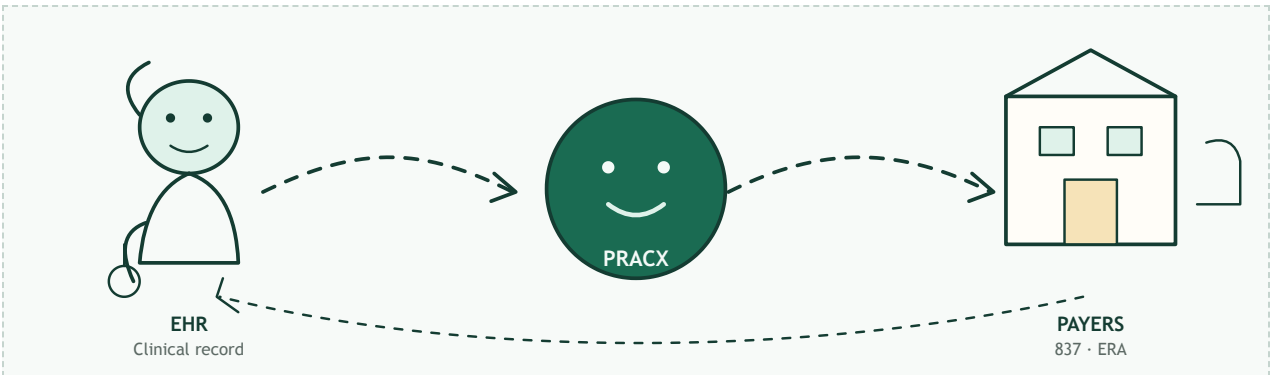
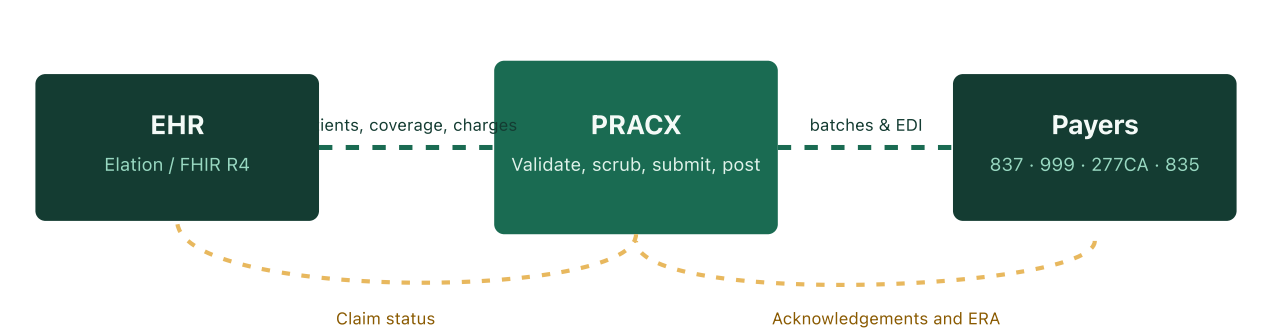


Figure 2. EHR, PRACX, and payer connections. The lower path returns acknowledgements, ERA, and claim status to the clinical system.



Direction	Data exchanged
EHR → PRACX	Patients, coverage, appointments, and encounters submitted as a FHIR R4 Bundle or PRACX inbound bundle v1. The dashboard action is Validate and queue. Accept applies records into PRACX; Hold and Reject do not.
PRACX → Stedi	837 professional claims, batch and EDI references, payer routing from practice setup.

Direction	Data exchanged
Stedi → PRACX	999 and 277CA acknowledgements plus 835 ERA for matching and posting.
PRACX → EHR	Claim created, submitted, acknowledged, paid, or denied, so the EHR remains current.

What the product does today

Capability	Current behavior
Inbound intake	Operations dashboard accepts FHIR R4 or PRACX v1 JSON. FHIR is converted to the PRACX bundle, then validated. Patient and Appointment are required. Practitioner must carry a US NPI. Coverage and Encounter are optional.
Queue status	Pending when the payload is clean or has warnings only. Held when validation has errors. Missing integration when no connector is configured for the source system.
Accept	Creates or links patient (by external ID or name and date of birth), coverage (creates payer and plan if needed), appointment, and a draft encounter. Stores applied PRACX identifiers on the inbound event.
Identity map	External EHR identifiers are stored against internal PRACX identifiers so subsequent events update the same patient, coverage, appointment, or encounter.
Provider and facility	Must already exist in PRACX. Mapping is by provider ID or NPI and by facility ID, NPI, or code. Missing maps hold the event; they do not create new providers.
Connectors	Elation API and Stedi EDI credentials are stored with test versus live mode. Live is blocked until required secrets exist. Last-test status is shown on Integrations. Posted bundles are the working intake path; connector credentials prepare live pull and EDI.
Eligibility	270/271 check from the patient or eligibility workspace. Returned address and coverage changes are applied only after staff choose keep current or use eligibility response and record a reason.

Inbound validation

PRACX does not create a claim from an unvalidated payload. On the integrations dashboard, staff submit a FHIR R4 Bundle or a PRACX inbound bundle v1 (event types: day appointment, patient demographics, insurance, encounter). The system stores the payload and a validation issue list. Required patient fields include name, date of birth, and address. Appointment start time is required. Coverage, when present, requires payer name, plan name, and member ID.

Accept is the only action that writes into PRACX. If apply fails, the event remains held with reason `apply_failed`. `Missing_integration` is corrected by configuring a connector whose source system matches the bundle, not by accepting the event.

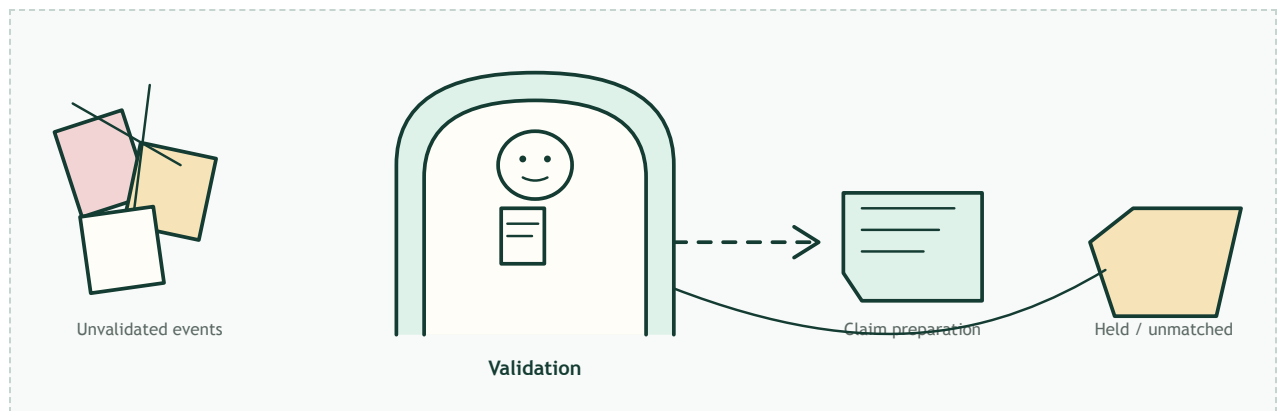
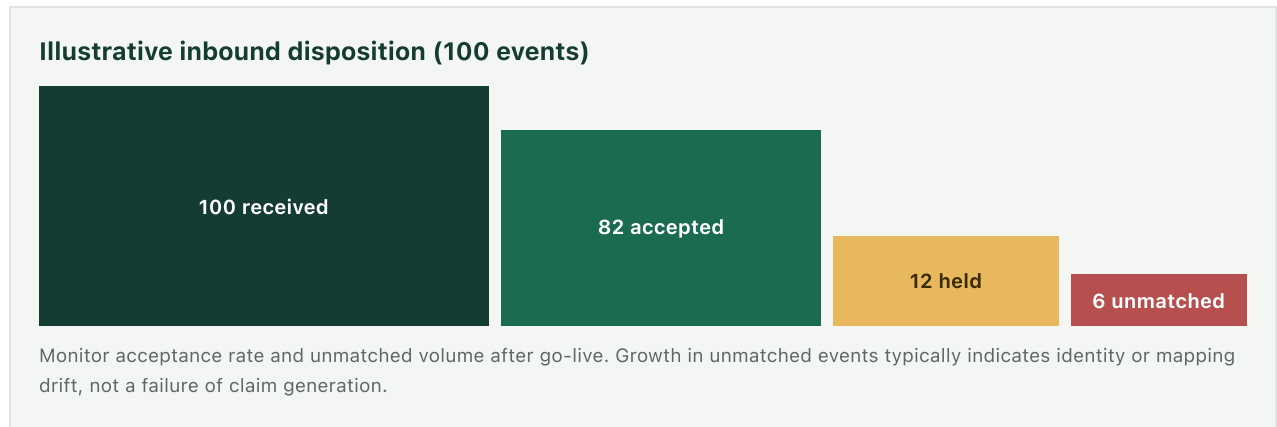


Figure 3. Inbound events are validated before claim creation. Exceptions remain in intake until resolved.



Claim lifecycle

Accepted encounters proceed through claim scrubbing, generation of 837 professional or CMS-1500, submission, payer acknowledgement, and ERA posting.

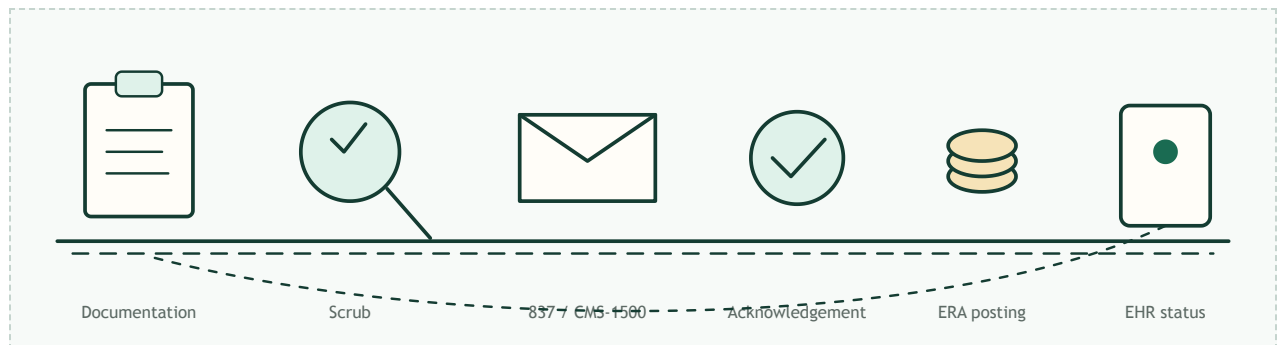


Figure 4. Claim flow from clinical documentation through submission, acknowledgement, posting, and status update.



Workflow statuses: needs scrubbing, scrubbing, error or ready to bill, generating, generated, submitted, then ERA posting. Blocking errors remain in queue and are not transmitted.

Eligibility verification

Automated or on-demand 270/271 checks at scheduling or check-in confirm active coverage, copay, deductible remaining, and coinsurance before service. That reduces eligibility-related denials and unexpected patient balances.

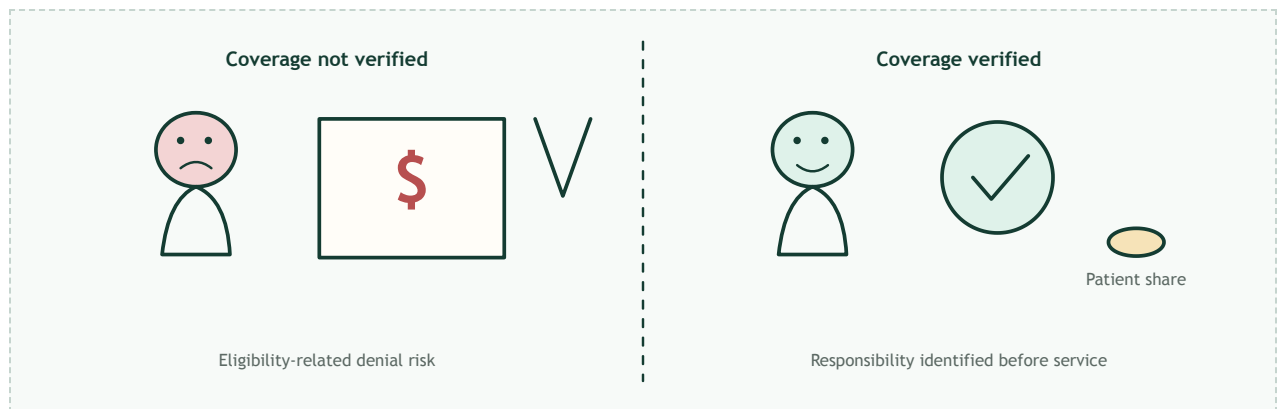
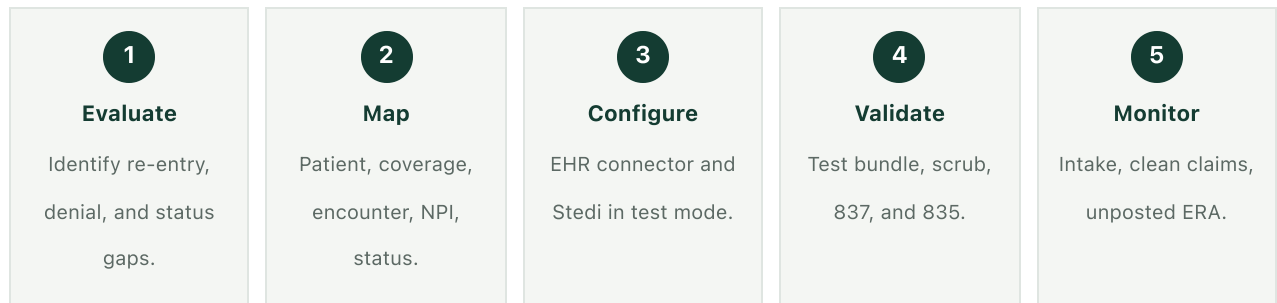


Figure 5. Real-time eligibility verification reduces unexpected balances and inactive-coverage denials.

Supporting capabilities

- **Coding assistance.** Office-visit CPT may be suggested from Assessment complexity (problem count, severity, status, differentials, plan rationale, diagnostic notes), taking the higher of time-based and MDM-style leveling. Codes are confirmed in Charges before they appear on the claim.
- **Claim scrubbing.** Errors and warnings are mapped to CMS-1500 fields, including patient, insurance, NPI, date of service, CPT, ICD-10, modifiers, and timely filing.
- **Payment posting.** Checks, EFT, 835 ERA, and paper EOB can be allocated across claims. Matching totals may post automatically.
- **Care operations (optional surface).** SOAP with structured Assessment (summary, working diagnoses, differentials, plan rationale), head-to-toe Objective exam, vitals, dictation, HPI assist, and encounter draft autosave that does not overwrite signed, ready-to-bill, or billed visits.
- **Same-day documents.** Photo or file upload, view, and AI heads-up. Drafts copy into diagnostic notes only when the clinician accepts. They are not written into a signed note automatically.
- **Patient chart.** Facesheet, history, problems, medications, immunizations, allergies, vitals, notes, labs and studies, flowsheets, demographics, account, care checklist, documents, and recall.

Implementation



Step 1 — Evaluate

Document where demographics, coverage, charges, and claim status reside today. Common gaps include paper charge tickets, eligibility checked only in a payer portal, ERA posted in a spreadsheet, and no claim status in the EHR.

Step 2 — Map data exchange

Define inbound event types: day appointment, patient demographics, insurance, and encounter. For FHIR R4, include Patient and Appointment at minimum; add Coverage and Encounter when available. Practitioner identifiers must use the US NPI system. Provider NPI and facility NPI or code must match records already in PRACX setup.

Step 3 — Configure connectors

Configure the Elation API or FHIR OAuth connector and the Stedi EDI connector. Add 270/271 and 835 adapters if they are not included in the same partnership. Align electronic payer identifiers. Remain in test mode until the last connectivity test succeeds and required credentials are present. Live mode is blocked until those credentials exist.

Step 4 — Test and train

On the dashboard, submit a test FHIR or PRACX bundle with Validate and queue. Confirm pending versus held and the validation issue list. Accept a clean event and verify that a patient, coverage, appointment, and draft encounter exist, then proceed to claim scrub, 837 or CMS-1500, and a test 835. Train billing staff on Accept, Hold, and Reject. Train clinicians that coding suggestions and AI heads-up require explicit acceptance.

Step 5 — Monitor

Track connector health, open inbound events, open sync items, clean-claim rate, rejected claims, and unposted ERA. Unmatched identity events are an early indicator of mapping drift and should be resolved promptly.

Integration challenges and PRACX controls

Observation	Likely cause	Control
Status missing_integration	sourceSystem does not match a configured connector	Configure Elation or FHIR under Integrations; do not Accept
Accept fails; event returns to held	Provider or facility NPI is not in PRACX, or payload is incomplete	Add the provider or facility in setup, correct the bundle, then Accept
Rising unmatched inbound volume	Name, date of birth, or member ID mismatch	Hold or reject; correct the source record; then accept
Scrub errors before payer submission	Incomplete or inconsistent claim data	CMS-1500 field-level scrub; coding confirmation
One-way feed; stale claim status in the EHR	No return path configured	Four-direction sync: EHR→PRACX, PRACX→Stedi, Stedi→PRACX, PRACX→EHR
Live connector failure	Missing credential or incorrect environment	Test versus live mode, last-test status, encrypted secrets

Frequently asked questions

Does PRACX replace the customer's EHR?

No. PRACX connects to the EHR the practice already operates. Elation is the primary named connector. FHIR R4 and the PRACX inbound bundle support additional sources when mapped.

Will integration reduce denials?

It reduces denials caused by transcription errors, inactive coverage, and incomplete CMS-1500 data. Medical-necessity and contract denials still require clinical documentation and payer follow-up; they become visible sooner.

What happens if inbound data is incorrect?

Do not Accept. Hold or Reject, correct mapping or the source system, then resubmit or Accept. Accept writes patient, coverage, appointment, and a draft encounter; it is not a status-only action.

Must the EHR post FHIR, or can it use a simpler payload?

Either. FHIR R4 Bundles are converted internally. PRACX inbound bundle v1 is a JSON contract with schemaVersion 1, sourceSystem, eventType, patient, appointment, optional coverage and encounter, and mapping for provider and facility.

How long does an integration take?

Duration depends on the EHR and the completeness of payer and provider setup. The sequence is: configure connectors in test, submit a test bundle, Accept and confirm applied records, scrub and submit a test claim, process a test 835, then move to live and monitor intake and sync.

Customer positioning

The practice retains its EHR. PRACX receives the data required to bill, validates and files claims electronically or on CMS-1500, posts ERA, and returns claim status so clinical staff have an accurate view of reimbursement.

Implementation begins with connector configuration and inbound intake. Claim preparation confirms that billing data is complete. Payments and reports confirm that the integration is operating as designed.

PRACX internal briefing. Diagrams are schematic. Chart values are planning estimates for internal discussion and are not service-level commitments. Product as of September 2026: FHIR R4 and PRACX v1 ingest with Validate and queue; Accept applies patient, coverage, appointment, and draft encounter; Elation and Stedi connectors; 270/271 eligibility with audited updates; 835 ERA; claim scrubbing; CMS-1500; optional care-operations chart, Assessment workspace, and document AI heads-up.